

The strategy in one line: the QR code does every sales conversation. Physical QR kits are couriered to clinics, digital QR/click-to-WhatsApp ads run online, and every conversation after a scan happens on WhatsApp or a screen-share call. **You never visit a clinic. No door-to-door. No events. No meetings.**

₹ The ₹1,00,000/month milestone — exact math

MediBook's live pricing: **Starter ₹0** (3 doctors, 200 appointments/mo — the hook), **Growth ₹1,999/mo**, **Professional ₹4,999/mo**, Enterprise ₹9,999/mo. The milestone needs roughly:

40 × ₹1,999 GROWTH CLINICS = ₹79,960	4 × ₹4,999 PROFESSIONAL = ₹19,996	≈ 44 PAYING CLINICS TOTAL	₹99,956 MRR ≈ ₹1 LAKH
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Bengaluru has an estimated **4,000+ dental clinics** — the milestone needs about **1% of them**. A one-time remote onboarding fee (**₹2,999**, waived during launch offers) adds cash on top of MRR while the base builds. Timeline at the funnel below: **~6 months**.

1 The three QR channels (all remote)

CHANNEL	HOW IT WORKS	TRACKING CODE
 QR kits by courier	A5 card (QR + "Book a dental appointment in 30 seconds — try it"), a short letter addressed to the doctor, and a pricing card — couriered to clinics via DTDC/India Post/Porter. Reception opens it; the doctor scans it.	?text=Hi K## per batch
 Digital QR / ads	Meta click-to-WhatsApp ads (Bengaluru 25 km, dentist/clinic-owner targeting) + a 60-sec demo reel on Instagram/LinkedIn/YouTube ending on the QR + Google Business Profile with the QR.	?text=Hi A1
 Referral QR loop	Every live clinic gets a patient-facing standee. Its footer carries a second small QR: "Clinic owner? Get this for your clinic." Dentists visit each other's clinics — the standee sells for you. Plus dealer/lab partners recruited by phone, mailed 20 cards each, paid ₹1,000/signed clinic by UPI.	?text=Hi R-slug / P##

Attribution is built into the product: each QR pre-fills a different first message (wa.me/91XXXXXXXXXX?text=Hi K12). The bot logs every inbound message, so scans-per-batch, per-ad and per-referrer are read straight from the dashboard — no analytics tooling needed.

2 Funnel assumptions (per 100 courier kits)

STAGE	RATE (ASSUMED)	COUNT	YOUR ACTION (REMOTE ONLY)
Kit delivered → QR scanned	~15%	15	Nothing — the card works alone
Scanned → completes demo booking	~60%	9	Bot does it; you get the log
Demo → replies to your follow-up	~50%	4–5	Personal WhatsApp within 2 hrs of scan
Reply → free trial (Starter) live	~50%	2–3	Remote onboarding same day
Trial → paying (Growth)	~40–50%	≈ 1	Razorpay link on WhatsApp

≈ **1 paying clinic per 100 kits** (≈ ₹6,000 kit cost per acquisition, recovered in month 3 of a ₹1,999 subscription). Ads and the referral loop compound on top — referrals become the cheapest channel by Month 4. These are planning assumptions: **measure weekly, reallocate ruthlessly**.

Rules of the game: ① No clinic visits, ever — courier, WhatsApp, phone and screen-share only. ② One personal message per clinic, replies only after that — no bulk blasts (keeps the number safe and reputation clean). ③ Every rupee of spend must map to a QR code you can attribute. ④ The receptionist is the real user — sell to her phone-load pain.



Days 1–7 — Foundation Week

Build the machine before spending a rupee on reach

PHASE 0 • SETUP

DAY	FOCUS	ACTIONS	DONE WHEN
Day 1	WhatsApp number + Meta	Buy a fresh SIM (never used with WhatsApp). Create Meta app → add WhatsApp product → add number, display name "MediBook Appointments". Submit Meta Business verification immediately (GST/business docs — takes 2 days–2 weeks; it gates everything, so it goes first). Start Railway deploy in parallel.	Verification submitted; dev-mode number live
Day 2	Deploy + demo clinic	Finish Railway deploy (backend, frontend, Postgres, Redis ≈ ₹1,000–1,500/mo). Register webhook, verify token, subscribe to messages. Create tenant " Sunrise Dental (Demo) ": 2 doctors, dental services (Consultation ₹300, Scaling, RCT Sitting 1, Braces Consult), timings 10:00–13:30 / 17:00–20:30.	You book an appointment end-to-end from your own phone
Day 3	Money + web presence	Apply for Razorpay (payment links + subscriptions). One-page landing site: headline, 60-sec demo placeholder, the QR, and a "Get MediBook for your clinic" form → your WhatsApp. Create Google Business Profile ("MediBook — WhatsApp booking for clinics, Bengaluru").	Landing page live; Razorpay KYC submitted
Day 4	QR system + print design	Generate the QR set with tracking codes: K01...K20 (kit batches), A1 (ads), P01... (partners), R-* (referral standees). Design in Canva: A5 QR card, doctor letter (100 words, names the receptionist pain), pricing card, reception sticker. Order 300 sets from a local printer (~₹2,500) — delivery, not pickup, if possible.	Print order placed; every QR tested on 2 phones
Day 5	Target list #1 (200 clinics)	Google Maps, area by area — Indiranagar, Koramangala, HSR, BTM, Jayanagar, JP Nagar, Malleshwaram, Whitefield: clinic name, doctor name, full address + pincode, phone, rating, review count. Prioritise 2–10-doctor clinics with active profiles (they already invest in patient acquisition).	Sheet of 200 with priority tags A/B/C
Day 6	Demo video + message system	Screen-record 60 seconds: patient books on WhatsApp → dashboard shows the booking → reminder goes out. Set up WhatsApp Business profile + quick replies for the whole cadence (post-scan, D+1, D+3, D+7 — scripts on Page 6). Submit any needed Meta template approvals.	Video sendable; every reply is one tap
Day 7	Dry run + kit assembly	Full rehearsal: scan → book → cancel → rebook → reminder fires. Fix every awkward bot moment — a skeptical dentist holding the phone is the whole sale . Assemble the first 50 kits (card + letter + pricing + sticker in an envelope, batch code on each). Book courier pickup for Day 8.	50 kits sealed, labelled K01–K02; pickup booked

Meta approval is the critical path. Until business verification passes, the bot can message only 5 pre-approved test numbers — enough for rehearsal, not for strangers scanning. If it's still pending on Day 8, ship kits anyway: scans land as inbound messages (allowed), and you reply personally until approval clears.

Time budget: this plan assumes **~4 focused hours/day** alongside product work. Days 1–7 are heavier (6–8 hrs). Nothing in this plan requires leaving your desk except handing envelopes to a courier pickup.



DAY	FOCUS	ACTIONS
Day 8	Wave 1 ships	Courier kits #1–25 (Indiranagar + Koramangala, batch K01). Send each clinic the one-time heads-up WhatsApp (script P6): "a small card is on its way to your reception." Log everything in the tracker.
Day 9	Wave 1 completes	Kits #26–50 (K02). Build the scan tracker: batch code → scans → demos → replies (read from bot logs). Set a phone alert for inbound messages — reply to every scan within 2 hours , that's the conversion window.
Day 10	First follow-ups	D+2 check on K01: one polite WhatsApp per clinic — "did the card reach reception?" + demo video to anyone who engages. No second message to silence.
Day 11	Wave 2 ships	Kits #51–75 (HSR + BTM, K03). K02 follow-ups. Offer scanners a 10-minute screen-share (Meet/WhatsApp video) in the dentist lull: 1:00–2:30 pm or after 8:30 pm.
Day 12	First remote demos	Kits #76–100 (K04). Run the booked screen-shares: <i>they</i> scan and book while you watch, then you show the dashboard. Close with the pilot offer below.
Day 13	Pilot offer to warmest 5	Pilot deal: first month free + free onboarding → ₹1,999/mo after, cancel anytime. Onboard pilot #1 fully remote: Google Form intake (doctor list, timings, services photo) → tenant configured same day → send their patient-QR standee as a print-ready PDF.
Day 14	Weekly review #1	Scans/replies/demos per batch and per pincode. Kill dead areas, double down where scans came from. Assemble next 50 kits. Checkpoint: ≥10 scans, ≥3 conversations, 1 pilot live.
Day 15	Partner recruiting (remote)	Kits #101–125. Call 5 dental dealers/labs (found on Maps/IndiaMART): the partner offer (Page 6) — ₹1,000 per signed clinic, paid by UPI same week. Mail each interested partner 20 coded cards. No meetings.
Day 16	Digital QR goes live	Kits #126–150. Post the 60-sec demo reel on Instagram + LinkedIn + YouTube Shorts, QR at the end. Ask (by message) to share it in dentist WhatsApp groups / IDA Bengaluru circles you can reach remotely.
Day 17	Ads on	Meta click-to-WhatsApp campaign: ₹300/day, Bengaluru 25 km, dentist/clinic-owner interest stack, creative = the QR card + "Your patients book on WhatsApp. You do nothing." Lands on ?text=Hi A1.
Day 18	Cadence day	Work the D+1/D+3/D+7 follow-up queue top to bottom. Onboard pilot #2. Tighten whatever confused people in demos — every demo is product QA.
Day 19	Wave 3 + proof	Kits #151–175 (Jayanagar + JP Nagar). Ask pilot #1 for a WhatsApp voice-note testimonial ("what changed at your front desk?"). Voice notes from a real dentist outsell any brochure.
Day 20	Warm-lead sprint	Every clinic that scanned but stalled gets one message: testimonial voice note + "the first-month-free launch offer closes Sunday." Deadlines move dentists; silence after that means not-now, recycle in 60 days.
Day 21	Weekly review #2	Per-channel cost per conversation (kits vs ads). Adjust ad audience; reorder print if needed. Checkpoint: 2–3 pilots live, first paid conversion in sight, ~₹2,999 collected in onboarding fees.

Why pilots convert here: the Starter plan is genuinely free, so "try it" has zero risk — but it caps at 200 appointments/month and 3 doctors. A clinic that uses it hits the ceiling on its own; the upgrade conversation starts from their dashboard, not from you.



Days 22–30 — First Revenue, Then the Loop

Close the first payers and lock the repeatable daily rhythm

PHASE 2 • REVENUE

DAY	FOCUS	ACTIONS
Day 22	Wave 4 ships	Kits #176–225 to the two best-performing pincodes only. Follow-up queue. One screen-share demo slot held at 1 pm and one at 8:30 pm daily from here on.
Day 23	Referral loop starts	Pilot clinics' patient standees go live (they print your PDF, or you courier a printed standee ~₹150). Footer QR: " <i>Clinic owner? Scan to get this for your clinic.</i> " — every waiting room becomes a billboard to visiting dentists.
Day 24	Cadence + partners	Follow-up queue. Check in with the 5 partners (WhatsApp): cards handed out? Restock by mail. First partner-code scans should appear this week.
Day 25	First paid conversions	Pilots whose free month ends: send the Razorpay subscription link on WhatsApp with a one-line summary of <i>their own numbers</i> — "You took 63 bookings this month; 9 were after clinic hours." The dashboard sells the renewal.
Day 26	Upsell sizing	Any clinic with 4+ doctors or >200 appointments → propose Professional ₹4,999 on the quota argument, not features. Kits #226–250.
Day 27	Cadence day	Full follow-up queue. Recycle Day-20 "not now" clinics that scanned again (the tracker shows re-scans). Ask every paying clinic for one referral : "which dentist friend should get this card?" — then <i>you courier the card</i> , they just make an intro message.
Day 28	Weekly review #3	Channel economics: ₹/paying clinic from kits vs ads vs partners. Set Month-2 print volume and ad budget from data, not hope.
Day 29	Month-end close	Chase every open Razorpay link (one reminder each). Reconcile: MRR, one-time fees, spend. Update the funnel-assumption table on Page 1 with your real numbers.
Day 30	Milestone gate	Target: 3 paying clinics ≈ ₹6k MRR + ₹6–9k onboarding fees collected, 3–4 more in trial. Below half of that → see the triggers on Page 5 before spending Month-2 budget.



The daily operating loop (Months 2–6, Mon–Sat, ~4 hrs)

SLOT	ACTIVITY
09:30–10:30	Follow-up queue (D+1 / D+3 / D+7). Reply to overnight scans first — speed is conversion.
10:30–11:30	Ship the day's kits (courier pickup Tue/Fri) · partner restocks · list building for next wave.
13:00–14:00	Demo slot 1 — dentists' lunch lull. Screen-share, their phone does the booking.
17:00–18:00	Onboarding + support: new tenant setup, receptionist check-ins (churn control), product fixes.
20:30–21:15	Demo slot 2 — after last patient. Close warm leads, send Razorpay links.

Churn is the silent killer of the ₹1L math. One 5-minute WhatsApp check-in per clinic per month with the *receptionist* (the real daily user) + same-day fixes keeps churn near 2–3%. At 44 clinics that's ~2 hrs/week — the highest-ROI slot in the whole calendar.



Months 2–6 — Scaling to the Milestone

Same daily loop, rising volume, compounding referrals

PHASE 3 · SCALE

MONTH	VOLUME & EMPHASIS	NEW PAYING	TOTAL PAYING	MRR (₹)	+ ONE-TIME FEES (₹)
M1	250 kits · ads from D17 (₹4k) · pilots	3	3	6,000	6,000
M2	400 kits · ads ₹9k · 10 partners active	5	8	16,000	15,000
M3	500 kits · ads ₹12k · referral loop visible	8	16	33,000	24,000
M4	600 kits · ads ₹15k · first 2 Professional	10	26	55,000	30,000
M5	600 kits · referrals ≈ 1/3 of new adds	11	36	76,000	33,000
M6	600 kits · 4–5 Professional in the mix	12 (–2 churn)	44–46	≈ 1,00,000	36,000



Monthly spend at peak (M4–M6)

ITEM	₹ / MONTH
Print + courier (600 kits × ~₹60)	36,000
Meta ads	15,000
Partner payouts (~5 × ₹1,000)	5,000
Infra (Railway) + tools	3,000
Total	≈ 59,000

Honesty box: ₹1L MRR lands ~Month 6; ₹1L net lands ~Month 8, as referrals (near-zero CAC) replace kit volume and spend drops toward ₹25k while MRR keeps climbing.



The only 6 numbers to track weekly

KPI	HEALTHY
Kit → scan rate	≥ 12%
Scan → reply-to-you rate	≥ 40%
Demo → trial rate	≥ 40%
Trial → paid rate	≥ 40%
₹ per paying clinic (blended CAC)	≤ 6,000
Monthly churn	≤ 3%



If-behind triggers (check every Sunday)

SYMPTOM	MOVE
Scan rate < 8% after 200 kits	The card is the problem: bigger QR, one line of copy, hand-written doctor name on the envelope (opens double). A/B two cards per batch.
Scans fine, replies dead	Your follow-up is too salesy or too slow. Reply within 2 hrs, lead with <i>their</i> demo booking ("you just booked with Dr. Sharma — that's what your patients get").
Trials stall unconverted	They're not hitting the Starter cap — help them push their patient QR standee harder (their usage is your revenue).
Ads CAC > 2× kit CAC	Kill ads, move budget to kits + partner payouts. Revisit ads at M4 with testimonial creative.
Any month misses adds by >50%	Freeze spend growth; spend the month converting the existing scanned-but-silent pool (it's hundreds of clinics by M3) before buying new reach.

Where the leverage compounds: by Month 4 every paying clinic carries a patient standee with your referral QR inside it. 26 waiting rooms × every visiting locum, pharma rep and dentist-friend = a permanent, free, growing billboard network. This is the channel that eventually makes the courier kits optional.



P6-1 — HEADS-UP MESSAGE (ONCE, BEFORE THE KIT LANDS)

"Namaskara Doctor, I'm Sandeep from MediBook, Bengaluru. A small card is on its way to your reception — it's a QR code that shows, in 30 seconds, how your patients could book appointments on WhatsApp. No app, no setup. If it interests you, just scan it; if not, please ignore this message — I won't message again."

Rule: one clinic, one message. Replies only after that. This keeps the number clean and is the entire anti-spam policy.

P6-2 — WITHIN 2 HOURS OF A SCAN (THEY JUST BOOKED A DEMO APPOINTMENT)

"Doctor, you just booked with Dr. Sharma at Sunrise Dental 😊 — that exact 30-second flow is what your patients would get, on the WhatsApp they already use. Here's the other side: [60-sec dashboard video]. Want me to set up a free trial for your clinic? You send doctor names + timings, it's live the same day — nothing to install."

P6-3 — FOLLOW-UP CADENCE (STOP AFTER THREE)

D+1: "Any questions about what you saw yesterday, Doctor? Happy to show the clinic-side dashboard on a 10-minute screen call — 1 pm or after 8:30 pm, whichever suits."

D+3: [pilot testimonial voice note] + "This is Dr. [name] from [area] after their first month."

D+7: "Last one from me, Doctor 🙏 — the first-month-free offer closes Sunday. After that I'll assume it's a no for now and won't disturb you."

P6-4 — THE OBJECTION ("MY PATIENTS JUST CALL")

"They'll keep calling — this catches the bookings you never see: the 9 pm WhatsApp when the clinic is closed, and the ones your receptionist misses mid-procedure. Clinics on MediBook see a meaningful share of bookings arrive outside clinic hours. It adds a channel; it changes nothing you do today."

P6-5 — REMOTE CLOSE (PAYMENT)

"Sending the payment link here — ₹1,999/month, cancel anytime from your side. The moment it's paid, your booking QR and reception standee PDF come right back in this chat."

Razorpay subscription link → auto-renewal. Onboarding fee ₹2,999 applies from Month 2 signups (waived in launch offers) — you do the whole setup, they photograph their timings board.

P6-6 — PARTNER RECRUIT (PHONE CALL, THEN POST)

"You visit 15 clinics a week anyway. I'll post you 20 QR cards — when a clinic complains about front-desk call load, leave one card. If they sign up, ₹1,000 to your UPI the same week. No selling, just the card."

Cards are coded (P01...) so payouts are automatic from the scan log. Pay fast once and they hand out cards forever.

✕ Deliberately not doing

CHANNEL	WHY IT'S OUT
Clinic visits / door-to-door / events	Out of scope by design — the plan is 100% remote. The QR + screen-share replaces every in-person step.
WhatsApp bulk blasts	One heads-up message per clinic, ever. Blasts risk the number and the brand — the number IS the product.
Cold email	Clinic inboxes aren't read by decision-makers. Use email only to send invoices and standee PDFs.
Justdial / IndiaMART listings	Low-intent traffic; the courier kit out-converts it per rupee.

The one thing to internalize: a dentist who scans the QR and books a fake appointment has already experienced the product's entire value. Everything in this plan — kits, ads, standees, partners — exists only to cause that one scan. Protect the demo's polish above everything else.